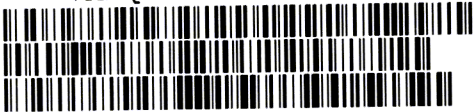


Report

---- SURGICAL PATHOLOGY ----

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TCGA-QG-A5Z1-01A-PR

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SURGICAL PATHOLOGY

PORT

Accession No.

Date obtained:

Specimen (Received

- A. LOW ANTERIOR RESECTION/STITCH MARKS PROXIMAL MARGIN
- B. PROXIMAL DONUT
- C. DISTAL DONUT

BRIEF CLINICAL HISTORY:

PREOPERATIVE DIAGNOSIS:
RECTO-SIGMOID ADENOCARCINOMA

OPERATIVE FINDINGS:

POSTOPERATIVE DIAGNOSIS:
RECTO-SIGMOID ADENOCARCINOMA

Surgeon/physician:

PATHOLOGY REPORT

Laboratory:

GROSS DESCRIPTION:

A. The specimen consists of 21.0 cm in length segment of colon, stapled at both ends with moderate amounts of attached fat. The specimen is received fresh to the frozen section lab for intraoperative gross consultation. Intraoperative findings are communicated to the surgeon in person. The serosa is tan-pink, smooth and glistening. There is a firm area of puckering in the mid portion of the serosa. The serosal aspect with the puckering is inked blue.

Opening reveals a 2.8 x 2.5 cm, tan-pink, centrally ulcerated mass with raised borders that is nearly circumferential, 9.5 cm from the proximal resection margin, and 5.8 cm from the distal resection margin. Sectioning through the mass reveals a tan-white, granular cut surface that grossly extends into the underlying attached mesenteric fat. The mass does not grossly extend to the inked serosal aspect. The mass is 9.3 cm from the mesenteric resection margin.

The remaining colonic mucosa is tan-brown, smooth and glistening. No subsequent lesions are identified. Sectioning through the attached fat reveals multiple tan-pink, rubbery lymph nodes ranging from 0.3 to 0.8 cm in greatest dimensions.

Representative sections are submitted as follows: A1- proximal resection margin; A2- distal resection margin; A3-A4- mass to closest inked serosa; A5-A6- mass to include underlying fat and adjacent uninvolved mucosa; A7- mesenteric resection margin closest to mass; A8-A13- multiple whole lymph nodes.

GROSS DIAGNOSIS:

RECTOSIGMOID COLON RESECTION:

- TUMOR IDENTIFIED WITH MARGINS APPEARING GROSSLY FAR FROM THE MASS.

B. The specimen consists of a 1.8 x 1.0 x 0.6 cm, tan-brown, annular soft tissue partially surface by a tan-brown, smooth and glistening mucosa. Sectioning reveals a tan-brown, unremarkable cut surface. No

Adenocarcinoma N65 8/4/13
Site CCF Sigmoid colon Q18.7
path Rectosigmoid colon Q19.9
Q10 4/16/13

discrete lesions are identified. Representative sections are submitted in cassette B.

C. The specimen consists of a 2.1 x 1.3 x 0.6 cm, tan-brown, annular soft tissue partially surface by a tan-brown, smooth and glistening mucosa. Sectioning reveals a tan-brown, unremarkable cut surface. No discrete lesions are identified. Representative sections are submitted in cassette C.

MICROSCOPIC EXAM

FINAL DIAGNOSIS:

A. LOW ANTERIOR COLON, RESECTION:

- SPECIMEN TYPE: RECTOSIGMOID, EN BLOC RESECTION.
- TUMOR SITE: RECTOSIGMOID COLON.
- TUMOR CONFIGURATION: INFILTRATIVE.
- TUMOR SIZE: 2.8 CM IN GREATEST DIMENSION.
- HISTOLOGIC TYPE: ADENOCARCINOMA
- HISTOLOGIC GRADE: WELL DIFFERENTIATED (G2).
- SURGICAL MARGINS
 - PROXIMAL COLORECTAL: NEGATIVE
 - DISTAL COLORECTAL: NEGATIVE
 - RADIAL: NEGATIVE
 - DISTANCE OF INVASIVE CARCINOMA FROM NEAREST SURGICAL MARGIN: 5.8 CM FROM DISTAL MARGIN.
- LYMPHATIC INVASION: PRESENT.
- VASCULAR INVASION: PRESENT.
- PERINEURAL INVASION: PRESENT.

PATHOLOGIC STAGING (pTN):

- PRIMARY TUMOR: pT3.
- REGIONAL LYMPH NODES: pN1b.
 - NUMBER EXAMINED: 38
 - NUMBER INVOLVED: 3

ADDITIONAL DIAGNOSES:

- TUBULAR ADENOMA ASSOCIATED WITH ADENOCARCINOMA.

B. PROXIMAL COLONIC DONUT, RESECTION:

- SEGMENT OF COLON, NO TUMOR SEEN.

C. DISTAL COLONIC DONUT, RESECTION:

- SEGMENT OF COLON, NO TUMOR SEEN.

COMMENT: THE TUMOR IS 0.5 CM FROM THE SEROSAL REGION IN FOCAL AREAS AWAY FROM THE SURGICAL SEROSAL MARGINS. THE ORIGINAL DIAGNOSIS FROM ALEXANDRIA WAS CORRELATED. DR. CONCURS WITH MALIGNANCY DIAGNOSIS. IMAGES ARE AVAILABLE IN VISTA. NOTIFIED VIA ALERT ON

MD

Signed

(End of report)

MD

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HPOA Discrepancy		
Prior Malignancy History		
Quasi/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	BRK	Date Reviewed: 2/27/13